



ARTICLE

Boundary Work

The production and consumption of health information and advice within service interactions between staff and callers to NHS Direct

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Abstract. This article explores the joint production and consumption of health information and advice in the context of NHS Direct, the new telephone health information and advice service. NHS Direct is an atypical call centre environment in so far as its core staff (nurses) are highly skilled professionals. These nurses are required to collaborate closely with, and are to some extent dependent upon, non-professional lower skilled staff (call handlers) to define and prioritize their work. This introduces a number of tensions between the two groups. Nurses respond to these tensions by engaging in 'boundary work', the micropolitical strategies through which work identities and occupational margins are negotiated, to protect their own status. Call handlers are more subject to the pressures of work 'speed-up', highly aware of the risks inherent in their role, but are unable to call upon professional status, either in their resistance to managerial imperatives or in their interactions with health consumers. They are required to manage the tension between offering a safe and high quality service, characterized in their training as 'professional', and nurses' boundary work, which positions them as 'non-professional'. Here we draw on qualitative research, including call interaction data between call handler and consumer, to show how call handlers' occupation of the contested territory between professional and non-professional status leaves them wide open to the agency of consumers, as consumers attempt to shape the service in their own interests.

Key words

call centres • call handlers • expert systems • nursing • occupational socialization • risk • telephone triage

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(London, Thousand Oaks, CA and New Delhi)
Vol 5(3): 315–337 1469-5405 [DOI: 10.1177/1469540505056793]
www.sagepublications.com

INTRODUCTION

Call centres might be seen as the iconic workplaces of the late 20th and early 21st centuries. Theorizing of call centres has focused on a number of issues, including: control; autonomy; resistance; quality and quantity; unionization; high commitment management; the inherent contradictions of service work; the viability of hybrid 'mass customization' models; and the effects of outsourcing on employees' organizational commitment (Ackroyd and Thompson, 1999; Batt and Moynihan, 2001; Deery and Walsh, 2001; Kinnie et al., 2000; Korczynski, 2001; Lankshear et al., 2001; Taylor and Bain, 2001; Taylor et al., 2002). Early studies tended to be characterized either by 'an abject pessimism . . . that sees call centres only in terms of technologies of surveillance' or by 'the uncalled-for optimism of the quality and virtual reality gurus' (Knights and Odih, 2002: 161). Those that come from a Foucauldian perspective on discipline at work (for example, Arkin, 1997; Fernie and Metcalfe, 1997; Sewell and Wilkinson, 1992) are tempered by studies that relate patterns of work in call centres to a variety of disciplinary technologies that reproduce existing inequalities at work within the wider society (for example, Knights and McCabe, 1998). The latter are more sanguine about the realistic, as opposed to the theoretical, potential of technology to control employees. Knights and Odih suggest that employers' dependence on emotional labour (Hochschild, 1983) and the social skills their staff deploy to ensure high levels of customer service mean that the employee relationship has to be managed extremely carefully, while Alferoff and Knights (2001) show how staff constrain and obstruct management attempts to deplete and systemize the know-how they have brought to their work. While earlier theoretical studies offer important insights, therefore, qualitative research approaches lend themselves to a deeper understanding of the call centre as a new workspace and of how workers in them learn to 'go on'.

This article arises from a research project entitled NHS Direct: Empowerment or Dependency?¹ The project aimed at addressing a number of questions around issues of risk, trust and reflexivity in service provision and 'consumption'. Giddens (1994) argues that we inhabit a post-traditional society wherein factors such as globalization and education alter the relationship between professionals and clients. Clients need expert advice more than ever, but, increasingly, they reflexively question this advice, thereby restructuring the nature of authority within the professional-client relationship. This leads to uncertainty and a shift away from traditional forms of production and consumption of professional medical services. In light of this, state and other service providers alter as individuals continuously question and challenge the nature of service provision.

However, although it appears that both distrust of experts and feelings of risk are growing, the work of Taylor-Gooby et al. (1999) suggests that the public still perceive the welfare state as the best mechanism for delivering services such as health care. They also wish to use the welfare state in traditional ways and seek security from it. Similarly, Hanlon and Jackson (1999) highlight the traditional way in which many individuals use professionals. In contrast with the public's desire for stability, it seems that in a risk society, governments are attempting to increase individual responsibility by developing new forms of service provision (Culpitt, 1999).

Our research located the activities of NHS Direct within these debates. It addressed questions such as: what are the people who use NHS Direct consuming and how is it produced? The Department of Health desires that NHS Direct provide a consistent service across time and space (something the NHS does not currently do). It claims that the service provides professional information and advice and allows people to manage self-care, all of which are increasingly important as traditional family and general practitioner (GP) relationships alter and undermine established medical service provision and advice giving. But, within this, a number of potential conflicts exist that may hamper the service's efficacy. It is important to know, therefore, what the calling public (the 'consumer') want from the service; what they receive; how they help create and then evaluate this process as reflexive actors; and how they use this information to interact with NHS Direct in the future, thereby helping to (re)create the service.

The project therefore addressed two key components, identified by Korczynski, of the debates around the relationship between service organizations and their 'customers': 1) how far customers' behaviour is determined by the structures of producing organizations or is informed by their own agency; and 2) the degree to which customers are formally rational or formally irrational (2002: 59). As he suggests, in practice, these are false dichotomies. We see customer conduct and service organizations being 'jointly produced' through the interaction between the two and our research methods reflected this view.

The research was conducted at two NHS Direct sites, one in London and the other a regional site serving a mixture of urban and rural populations. The populations served by the two sites are of similar sizes (3.8 million and 3.6 million, respectively), although the regional site employed more staff than the London one (full-time equivalent of around 70 nurses/36 call handlers, compared with 39 nurses/23 call handlers) due to the higher volume of calls handled by the regional site.² The employment backgrounds from which call handler recruits came included previous work

in the NHS in such areas as medical records; commercial call centre work; service occupations (such as bus drivers); and, in the regional site, in particular, a number of new graduates. Participant observation revealed considerable variation in their initial orientations towards public service work and towards notions of professionalism.

We used a variety of qualitative methods, including participant observation (one of the researchers undertook call handler training followed by a limited number of supervised shifts 'on the lines'); work shadowing; indepth interviews with both call centre staff and callers; and conversation analysis (CA) of recorded call interactions. The analysis presented here is based upon the participant and non-participant observations/shadowing at the call centres, interviews with nurses and call handlers and analysis of call interactions between staff and callers.

We illustrate how a complex set of organizational relations act to create space for the active intervention of the consumer in service production. We begin by classifying NHS Direct as a call centre. We outline the technologies that the service uses and the rationalizing functions these aim to perform. We go on to show how nurses subvert managerialist discourses of both consumer sovereignty and organizational efficiency in order to protect the boundaries of their own professional autonomy. In drawing upon professional knowledge and practice to respond to consumers' individual needs and circumstances, however, they also make the technology 'work'. Nurses also attempt to protect their status as professionals in another way. They are to some extent dependent upon call handlers to define and prioritize their work for them and they engage in informal 'boundary work' (Allen, 2000) practices with call handlers, which clearly position the latter as 'non-professional'. In order to show how call handlers deal with this, and what the implications of these tensions are for both consumers and for the service, we go on to describe the organizational socialization of call handlers. Caught between nurses' boundary work and the 'quasi-professional' discourse of their own training, call handlers become vulnerable to consumers' attempts to shape the service in ways that do not necessarily coincide with the expressed aims of the organization. We present call interaction data between call handlers and patients, which illustrate the consumer's contribution to service production. We conclude by discussing the implications of the findings for service production and consumption and for professional-consumer relations.

NHS DIRECT AS A CALL CENTRE

Designed to address a rising demand for emergency services for comparatively 'minor' ailments and to help individuals take responsibility for their health, the setting up of a new 'nurse-led' telephone advice line was announced in the 1997 White Paper *The New NHS: Modern, Dependable* (DoH, 1997). In it, the government declared its intention to modernize the NHS so that it could meet patients' aspirations for up-to-date, quicker and more responsive services. NHS Direct was described as one of the key developments in this. The service now employs approximately 1100 full-time equivalent nurses across its 22 sites in England (there is an additional site in Wales and three more in Scotland, where the service is called NHS 24) and between its launch in March 1998 and July 2003, handled over 18 million calls. One of the major aims of this new service was to enable people to care for themselves and their families at home through making best use of modern communications technology to provide clinical information and advice, helping them make the most appropriate use of NHS services. This nicely combines what Lupton calls the current 'imperative of health', to which the responsible citizen responds on an individual basis, through various 'rational practices' of an 'entrepreneurial self' (1995: 61), and the idea that the 'correct' information and advice will enable or 'empower' the public to make rational choices about their use of health services in ways that accord with a rational allocation of resources ('the most appropriate use of NHS services').

While the technology used by NHS Direct supports convenient and relatively fast access to health care for its 'consumers', it also performs a number of other functions: it allows for the same kinds of surveillance and work measurement practices as those used in other commercial call centres; it introduces new kinds of risks associated with what is referred to as 'triaging blind' (that is, making an assessment and advising the caller on the most appropriate course of action to take, without the kind of information that normally arises in face-to-face interactions, from being able to see and touch the patient); and it puts in place a clinical assessment system (CAS) and prioritization protocols to manage these risks.

The technology used by NHS Direct is an integrated telephony and computer system. Call handlers make initial decisions about the urgency of the call with the use of 'priority protocols'. They are able to connect callers directly to the ambulance service, if necessary, and assign a priority rating for less urgent calls to be triaged by nurses. Non-symptomatic health information queries are assigned the lowest grade and are dealt with by non-nursing health information advisors. Nurses take calls from the 'queue'

in priority order and ring the caller back. Over and above enabling this division of labour, technological functions include:

- a central electronic display of 'performance indicators', such as number of calls waiting to be answered;
- enhancing call centre supervisors' efforts to meet demand by monitoring when staff are 'on the lines' and 'off the lines' and to manage surges in call volume by re-routing calls to other sites at peak times;
- enabling nurse team leaders to monitor individuals' practices in relation to call length as part of 'quality control' measurement;
- enabling nurses to take time out at 'quiet' times to listen to recordings of their own calls in order to enhance their own 'professional development';
- the use of call recordings to address callers' complaints; and,
- extraction by administrative staff of statistics in order to monitor demand over time, to plan staffing, to analyse types of callers and reasons for calls and to submit site statistics to a national Health Information Unit.

These functions are familiar from studies of commercial call centres and appear to be a long way away from management texts, which link quality to what Davies and Kirkpatrick refer to as 'the empowerment of street-level staffs' (1995: 104). What is distinctive about NHS Direct, however, is the fact that its core staff (nurses) are highly trained skilled professionals rather than low-skilled low-paid workers. One effect of this is the way in which discourses of managerial control, delivery of high quality service, nurses' 'professionalism' and their professional development can shift, become conflated or be used ambiguously, according to context.

While there are no rigid rules about how long the nurse 'callback' is expected to take, call lengths are monitored and there are informal understandings of what is acceptable and what would provoke concern:

I think they would be concerned more if you were doing a huge volume of calls because they'd wonder then if you were getting the quality. . . . And also they'd be concerned if you were doing a lot less than everybody else because that might mean you'd got a training need . . . if you are sort of middle of the road and your average is three calls an hour and that's what you always do more or less . . . and what you're doing is good quality, then I don't think they're bothered. (N02)

NURSE PROFESSIONALISM AND RESISTANCE

Call length and rates are nevertheless a 'site of resistance' in which nurses call upon their status as professionals in a bid to define good practice, regardless of whether or not it contradicts other definitions of good practice, such as lessening the risk of deterioration of the patient's condition while waiting in the queue for a callback:

I think at the beginning most nurses are aware of it [the electronic display] and feel a bit pressurized and I think the call centre mentality could very much creep in if it was allowed to. Luckily, we're all very senior nurses. We are very much aware of our accountability to the patient and to ourselves and our profession. (N18)

Nurses do not only resist managerial discipline on the grounds of being accountable to their profession, however. They also position the emotional labour that is central to their nursing work (Bolton, 2001; James, 1992) as cost-effective on organizational grounds:

I did some call reflection and the team leader said, 'You're supposed to do six calls an hour, you're doing four'. And I went, 'Yes, what is your point?'. I said, 'Do you want quantity or quality? If you want quantity, you've got the wrong person. If you want quality, that's fine. Have you got a complaint with my calls? Are they thorough?' 'Very.' 'Fine, end of story.' And it's never been said to me again . . . the number of calls shouldn't be an issue because we're professional people and if I feel somebody needs half an hour, then that's what they get. . . . If I reassure that one caller on the phone, she gets good quality advice, she feels she can ring back at any time. That's saving an out of hours GP, that's saving a wait in Accident and Emergency, that's saving her time. I think it's cost-effective. (N01)

These are nice examples of staff not only being aware of the twin logics of routinization and efficiency, on the one hand, and of the customer and customer orientation, on the other, which are at work in the 'customer-oriented bureaucracy' (Korczynski, 2002: 4), but of nurses turning managerial discourses of both consumer sovereignty and efficiency 'against' management in order to protect their own autonomy.

NURSE PROFESSIONALISM AND THE 'EXPERT SYSTEM'

Nurses also draw on their professional knowledge and skills in giving advice to patients. Part of the training they receive when they join NHS Direct is in skills such as active listening and 'picture building' (Pettinari and Jessop, 2001) in order to offset the risks of 'triaging blind'. In addition, they are assisted in the assessments they make of patients' symptoms by CAS, which is also designed to minimize risk. It is built around a library of around 200 detailed algorithmic question and answer sequences. 'Final dispositions' include: Accident and Emergency, immediate or routine contact with a GP, advice on self-care at home, and information giving. Nurses are able to select a higher or lower disposition than that recommended, but must document their reasons for doing so. The claims for this technology are that: first, it minimizes risk by incorporating evidence-based rationales within the algorithms that underpin the advice-giving; second, it minimizes risk by the fact that the question and answer sequences are constructed to initially address the 'worst case scenario' (and to exclude or respond to it); and, third, the system claims to offer a *standardized* minimum safe level of service regardless of nurses' professional background or site location.

Our research shows, however, that nurses draw upon their professional and local knowledge, and that of their nursing colleagues, in order to 'make CAS work' (Greatbatch et al., 2005). Thus, for example, the nurses tailor their use of CAS to accommodate a number of human and organizational factors, including the precise nature of patients' symptoms, whether patients have already received medical assistance (and, if so, what this involved), patients' levels of knowledge about their symptoms, patients' familial and social circumstances, patients' levels of anxiety (as expressed in the calls), the nurses' own and their colleagues' specialist knowledge and expertise and the nurses' knowledge of how other local NHS services operate in practice. In addition, there are aspects of the professional expertise and reasoning of nurses that resist being transformed into rules that can be embodied in so-called experts systems like CAS. For instance, in triaging cases, nurses make a range of judgements, many of them tacit, about the callers and their circumstances. Is the caller exaggerating and/or correctly describing the symptoms? What does the caller want: reassurance, advice, access to other parts of the NHS or emotional support? Do callers understand what they are being told? In making these judgements, nurses rely on practices and procedures that are largely tacit and that are rarely discussed or even thought about. These kinds of judgements lie beyond the purview of expert systems.

The major aim of these technologies is to create a safe and rational system of health care allocation to large numbers of patients (with an

average annual number of calls answered of around 450,000 at one of our sites), in part by controlling and circumscribing nurses' work. One of the paradoxes of organizing the delivery of health care advice in this technologically mediated way is that, alongside the disciplinary practices it affords management, and despite the more onerous aspects of work it shares with other call centres, such as the inherent monotony (Knowles et al., 2002), it does leave space for some nurse autonomy, both in drawing on the expertise of a wide range of nursing colleagues, over and above CAS, and also by organizing contact with patients on an individual basis. The latter is experienced as enhancing nurses' autonomy by reproducing the kind of one-to-one nursing care that attracted many into the profession in the first place. Furthermore, they talk very positively about developing a whole new set of transferable communication skills associated with 'telephone consultation'.

Nevertheless, they remain dependent upon call handlers, who, in recording the 'reason for call' (in approximately six words) and allocating it a priority rating, are to some extent defining and prioritizing the nurses' work for them. Again, nurses respond to this by engaging in boundary work, which once more refers to their own professional status. At the same time, these practices throw into relief the scale of responsibility undertaken by the call handlers and raise their awareness of the risks they are required to manage as 'non-professionals'. The tensions they manage are evident in the partial 'socialization' of call handlers, via training and practice, into an ethos of 'quasi-professionalism', which has the potential for increasing rather than minimizing risk.

BECOMING A CALL HANDLER FOR NHS DIRECT

Shadowing call handlers quickly revealed the kind of responsibilities they had and some of the strategies used for dealing with them. For example, an experienced call handler demonstrated tailoring his responses to the verbal cues he picked up from callers. On one occasion, he moved from joking with one caller about her name, to the next, who was reluctant to give his home address because, it transpired, the caller was presenting with a suspected sexually transmitted disease. The call handler immediately became more serious, accepting and reassuring, telling the caller to give 'as much or as little detail as he wished' about his symptoms.

In the break between calls that followed, the call handler went on to describe, rather comically, the way some older callers called him 'doctor'. He would reiterate, 'I'm not a doctor actually, but I'll put your call through to a nurse who will call you back'. Then they'd reply, 'Oh, alright then

doctor'. 'What do they think', he asked rhetorically, 'that we've got banks of "knitting" doctors, sitting here waiting for calls? "Click, click, oooh, I'm getting bored, I do wish another call would come through, click, click!"' He talked of his joking as a way of relieving boredom, but it may have served a dual purpose, by also equating to the kind of 'gallows humour' routinely used by medical staff as a coping mechanism for dealing with distressing situations, especially since he also confessed to getting 'a bit depressed at first' listening to people's troubles and illnesses. Whatever purposes it served, he was certainly adept at assessing communication styles and utilizing differential responses to callers' expressions of need, in the way that nurses have been shown to do in their 'emotion work' (Bolton, 2001).

Recruitment criteria for call handlers at this site included not only a basic level of competence with computers and a facility for using telephone equipment, but an appropriate orientation towards callers who are *patients*, which latter criterion was stressed in training. Participant observation at a two-week training programme was undertaken alongside a new cohort of recruits. Early on in the training, the in-house trainer attempted to convey the message that, despite some of the documentation being used referring exclusively to 'nurses', call handlers should not think of themselves as lower status: 'You're very much a group of professionals, amongst other professional staff.' This was negated somewhat by the next training exercise, which required trainees to 'interview' other staff around the call centre about their roles. As one of the nurse interviewees explained how he took calls off the screen according to the priority rating they had been assigned, he added, 'which is a contentious issue'. Asked why, he was unequivocal: because it was a clinical decision and 'you're not really qualified to make that decision'. An experienced health information advisor's account of this issue, however, revealed a much more 'cooperative' picture, in which she had learnt from sitting alongside both kinds of colleagues and had built up a body of (medical) knowledge and experience. We will look later at what the implications might be for this kind of knowledge acquisition.

Other elements of training, undertaken by a team leader, related much more to contractual issues and to the call centre as a work environment. He went smartly through a list of 'rules': there was no dress code; drinks were allowed at work stations, but not food; no mobile phones were allowed in the call centre; the administration office telephone number was the one to be given out to family and friends who needed to contact staff at work, not the NHS Direct number; staff were not to engage in 'party atmosphere' talk within range of headsets; and help was at hand for 'challenging calls' after which it was okay to take some time off.

On the other hand, the ethos and organization of this site's training of call handlers did not adhere strictly to the national training 'curriculum', which had been developed and circulated and which was highly prescriptive about the required content and timetable of training programmes. Rather, training attempted to encourage an understanding of NHS Direct as part of the NHS and the development of skills that were compatible with this. It also constructed service consumers as in need and as deserving of skilful help. Examples of such messages include:

- not taking callers' words on face value, but *interpreting* what they are saying;
- demonstrating empathy with callers who may be frightened, anxious or feeling isolated;
- 'summarizing' what the caller says, not only as a call management strategy designed to move the caller on to the next stage of the call, but as a way of ensuring the accuracy of the information being recorded;
- a habit of reflective practice designed to enable call handlers to make sensitive judgements, in line with service policies, about how best to help patients; and,
- a public service ethos, communicated through a variety of statements made by the trainer, such as, 'You're only here because you have the right attitude – a caring attitude'; 'You're not going to give a good service unless you have good expectations of the caller, that they are a) honest and, b) trying to cope. You must take that away with you above all else . . . no matter what you feel about their anger or distress'; 'Try not to feel resentful [if they refuse to give their name and address] and still give good service, as this has an impact on them coming back to the service. For example, callers with a new and difficult diagnosis may be in shock and may refuse offers of information or offer of nurse advisor, but if you remain warm, professional and appropriate, research shows that they may well come back later when they've come to terms a bit more with the diagnosis. . . . So remember that you're helping them in a process.'

Here, the consumer is clearly being constructed as an individual patient in genuine need and the call handler as subscribing to and 'performing' an 'ethic of care' (Tyler and Taylor, 2001).

This approach to training, in contrast to the heavily 'prescribed' curriculum, was seen as *trusting* call handlers, 'empowering' them and treating them more like 'intelligent grown-ups', and they were recruited very much with these expectations in mind. It also left them exposed to

nurses' boundary work, however, in which they were defined *in opposition* to professional status. In this context, call prioritization was experienced as particularly difficult and the call handler became vulnerable to identifying closely with service consumers who had been constructed for them in training as both genuinely needy and deserving of help:

In the training, I think the hardest thing was . . . to know how to prioritize the calls. . . . There are so many protocols, but then when you are actually taking the call, so many of your emotions come in . . . it's hard to say, 'this is a number one and this is a number three'. . . . Especially when, to the caller, their illness is more important than anyone else. . . . Having children of my own, I always find it difficult when there's a young child, under three-ish, that's ill . . . to say, 'that's a "three"', that's got to wait two hours for a callback' . . . you're supposed to put it as a 'three', but I always find I put it as a 'one'. Just because the children are so young and especially as they can't say what's wrong with them. It's hard to say, 'Oh, they're fine, I'll put it through as a "three"'. They'll wait six hours . . . I had one where his wife had terminal cancer . . . and he wanted to see his doctor. He didn't want to speak to a nurse. . . . And it's hard to say, 'I know your wife's got terminal cancer, I know she's in pain, but this is the way your doctor has chosen it to be'. . . . He got very angry with me . . . when people have terminal illnesses, I always put it through as a 'one' because they're suffering, they have been diagnosed with a condition that means they suffer. Why should they wait for six hours? (call handler 2, three weeks post-training)

As time went on, however, some call handlers began to acquire pieces of medical knowledge, which they then used in their prioritization practice:

I'm aware of Warfarin and its importance and there's been incidents where people have perhaps called the service where they are having a nose bleed and they are actually on Warfarin. And they've been put into the queue for, say, three hours, but that's because that individual person doesn't know the importance of Warfarin. So certain circumstances or certain medications that people perhaps could be made more aware of. . . . As I say, I am aware of some medications and the importance of them, which would then indicate how –

prioritizing the calls really. (call handler 1, six months' experience)

Equally difficult at first for call handlers is to learn not to answer callers' health questions, even when they think they know the answer:

I've had a call before where an elderly lady went to an even elderlier lady's friend's house and she found some cocodamol in a drawer and she wanted to know what they were. And I just wanted to say, 'They are painkillers, just like aspirin or paracetamol'. But you can't . . . [it's] a hard thing to say, 'Well, I don't actually know', when you do. . . . I had this woman yesterday who hadn't been taking any contraception, she'd had unprotected sex, her period was three weeks late and she was taking some tablets for a condition – did I think that the tablets were causing it? I just wanted to say to her, 'Wake up!'. . . . That's the hard thing . . . people say things, you hear it in your ear and you're just like, 'Is this what you believe? You're telling me you're believing it's your tablets that's made your period three weeks late?!' (call handler 2, three weeks post-training)

But a sense of themselves as 'professional', with a 'responsibility' towards the caller and towards the service, clearly developed over time:

We are front line staff . . . we definitely try to be professional and letting them know what's happening. That is essential really, so then they feel that they've got faith in the service itself. [How do you do that?] Try and explain every step of the way. If they have a question about why this is happening or why that is happening, explain to them clearly: the nurse's role. If they ask why I am asking them certain questions, I try and explain to them why I'm asking these questions. (call handler 3, six months' experience)

We were just told, if you get an aggressive caller, tell them . . . 'It's all recorded and we would terminate the call, we would be unable to help you'. But I don't like to terminate calls because this gentleman or this lady had called for help. So they're panicking, they're frustrated, they've waited for 15 minutes most probably, in queue, and to listen to that message again and again, and people do get very fed-up, I can imagine. Obviously, when they've finally got through to whoever it is, they want to let

out steam, so you've got to be prepared to take that steam for a while because unfortunately it's part of the system, it's part of the job. You've got to sit there and think, 'okay', you've got to let that bit go over your head . . . it used to be [stressful], but I'm so used to it, it's not anymore. At first I used to worry. I'd think, 'Well, it's not my fault, I'm doing my bit'. But you think, 'You're part of the NHS and they're getting enough of it publicly, media-wise, I'm a part of the system that isn't, is not given its due'. (call handler 4, 12 months' experience)

Having been socialized into strong 'customer empathy', call handlers find it particularly difficult to deal with an irate caller (Korczynski, 2001), especially when they are only trying to 'do their bit', although here we see blame being displaced on to a 'knocking', unfair media.

Further down the line, very experienced call handlers became confident about adding a 'personal touch' without compromising their sense of 'professionalism'. Despite their nomenclature, they still saw themselves as public service personnel with a real pride in their work and they very clearly interacted with callers as *socially embedded* 'customers':

I hate the word call handler . . . I was told . . . that I've got to say that I was one of the 'advisers' because they wanted continuity across the board . . . I were quite proud of my title [assistant health information advisor] . . . the main part of our job is to make sure that the person at the other end realizes they are talking to a person and not a machine. That's how I look at it. . . . One of the first calls I ever took, a gentleman whose wife had hurt her back and the most comfortable position she had got was on the floor . . . she was in severe pain . . . the television was on and the music to *Who Wants to Be a Millionaire?* came on. And I said, 'You've got *Who Wants to Be a Millionaire?* on there?'. And he said, 'Yes, love, it's the only thing that's keeping her quiet'. And I said, 'Perhaps she thinks you're phoning a friend'. And he said, 'Oh my darling, you've made my night'. And his wife turned round and she started to laugh. And he said, 'That's the best thing I've seen all day, that, she's actually smiling'. And I thought, well, perhaps it was the wrong thing to say, but he realized I was human and, you know, I think people ring up with depression or they are suicidal, they need to know they are talking to somebody who is going to respond to them. (call handler 3, 18 months' experience)

Empathetic 'joking' like this, in the face of distressing circumstances, is a familiar part of the emotion work of nurses. Here was evidence that call handlers engaged in it too, that they developed a kind of quasi-professionalism, which involved the ability to create a rapport, to present a 'professional' face rather than a 'personal' one (which could – indeed, should – include 'human touches' to distinguish them from automata), to create confidence in the service, to use medical knowledge gleaned from working alongside nurses and to develop strategies to cope with angry or distressed callers.

Nevertheless, call handlers lacked the same recourse as nurses to an official discourse of professionalism as a mechanism of resistance and were therefore more vulnerable not only to demands placed upon them by service consumers, but also to managerial disciplinary measures. Sometimes they experienced difficulty in reconciling these tensions:

I kind of have had conflicting advice . . . I've had it on a day when the same call centre supervisor has said to me, 'Get back on line' and then, in other respects, on another day, I've discussed it with them further, about feeling that perhaps I'm not paying too much detail to callers . . . and I was told that really I ought to be taking more time and slowing down. I'm being told to speed up and take more calls and then I'm being told to slow down . . . and then being told, 'Don't look at the queue, don't watch the queue', but then you're being told, 'There's "so many" people in the queue, get back on line'. So your head is kind of spinning round in circles. (call handler 3, six months' experience)

Moreover, it appeared from their interviews that there was also a temptation for call handlers to stray into territory to which the nurses lay claim:

a lady has called and her husband is in the other room banging on the door. She said, 'I want to talk to somebody'. I said, 'Well, you need to call the police first of all, get yourself protected, secured, then call us back'. . . . She said, 'I need help, I've just been hit by my husband, what do I do? My nose is bleeding and everything.' I said, 'First of all, get rid of the trouble and obviously call us back or obviously when the police get there, they will look at you and take you straight to casualty'. . . . I had another one, a girl hit another girl in a fight. She said, 'I've just been hit and my nose just doesn't stop bleeding'. I said, 'Where do you live?'. She goes, 'Oh, so and so'. It was literally opposite

the hospital. I said, 'There's a hospital across the road'. She said, 'I don't want to go to hospital, it'll become a police case'. I said, 'Not necessarily'. I said, 'If you just go in there and say you've just had your nose, you know, hit by accident, a friend just headbutted you when you were just playing around', I said, 'it's up to you if you want to make a police complaint, but', I said, 'if it's broken, you need to go to get it looked at straightaway'. So she agreed to and went straightaway. I said, 'By the time I get you a nurse advisor and triage, we're so busy', I said, 'you'll be seen at casualty by then and at least they'll stop you bleeding'. (call handler 4, 12 months' experience)

Not only do the porous boundaries between professional nurse and non-professional call handler leave the latter already vulnerable to straying into nurse territory, but the active agency of the 'deserving' consumer within the call interaction itself can act to seduce or pressure the call handler into doing so, thus shaping the service in ways that do not necessarily coincide with the expressed aims and role prescriptions of the organization. The tensions surrounding the role and status of call handlers, and the part played by the consumer, are starkly illustrated in a recording of a call to one of our fieldwork sites in which a patient tells a call handler that she is in severe pain and is having difficulty coping, despite taking painkillers prescribed by her GP. Just prior to contacting NHS Direct, the caller has telephoned her GP because the painkillers he prescribed are, in her view, barely touching the pain. Apparently, the GP has informed her, via a receptionist, that the tablets she is taking are the strongest available, hence her call to NHS Direct.

When we played this recording to several nurses in workshops at both sites, they all claimed that the call handler acts inappropriately in the following part of the call:

1. Caller: uhhhhh The receptionist's just rung me back no::w after I rang first
2. thing this morning ((voice breaking)), [.h
3. AHIA: [Yea:h.
4. Caller: and said the doctor can't give me any stronger painkillers.
5. (.)
6. AHIA: Oh dea::r
7. Caller: It just doesn't seem right. I am desperate.
8. AHIA: Ri:ght
9. Caller: I haven't slept for four nights,
10. AHIA: No:

11. Caller: .hh[hh And I'm in agony.
12. AHIA: [Oh dear.
13. Caller: Wh[at am I supposed to do:?
14. (AHIA): [()-
15. AHIA: O:::h (.) dear.
16. (.)
17. AHIA: .hh Well I mean hh
18. (.)
19. Caller: hhh[hh
20. AHIA: [I don't know what to suggest. I can- I
21. can [obviously (.) get the nurse here to talk this=
22. Caller: [hhhh
23. AHIA: through with you: and er
24. Caller: hhhh.hh
25. AHIA: you know give you some advi:ce but if it's
26. down [to the GP: again (0.2) uh- you know it's difficult=
27. Caller [hhh.hh
28. AHIA: =isn't it if you- if your GP (.) won't prescribe you anything
[e:lse.
29. () : [()
30. Caller: Well he said I've go:t- they'd given me really strong stuff
yesterday
31. and I've [just got to- it's=
32. AHIA: [mhm
33. Caller: =going to take its ti:[me (but I mean) I've got two=
34. AHIA: [Yea::h
35. Caller: =kids to loo[k after..hhh hhh I'm good for noth[ing.

The nurses claimed that the call handler acts inappropriately in two important respects. First, in their opinion, the call handler fails to maintain an appropriate social distance between herself and the caller. Up to this point, apart from asking questions, the call handler has confined herself to producing a series of empathetically intoned acknowledgement tokens ('Right' and 'Yeah') as the patient explains her predicament. However, after the caller's voice starts to break as she reports her GP's unwillingness to prescribe stronger painkillers (lines 1–2 and 4) and then describes the severity of the problems she is experiencing (lines 7, 9, 11 and 13), the call handler upgrades her affiliation with the caller's plight ('Oh dear', lines 6, 12 and 15). According to the NHS Direct staff, the call handler fails to maintain the levels of affective neutrality expected of a medical 'professional'.

Second, the nurses expressed the view that the call handler encroaches on *their* professional territory by addressing clinical issues rather than merely prioritizing the case and arranging for a nurse to call back. Specifically, the call handler suggests that there may be little that a nurse could do, given that the GP will not prescribe anything else (lines 17–28). In the view of the nurses, the call handler was not qualified to comment on clinical issues in this way and should not have represented herself as having such a role.

This case illustrates some of the tensions surrounding the call handlers' role. The nurses expect the call handlers to act as medical professionals by refraining from adopting an 'everyday' footing in their dealings with callers who are apparently distressed, anxious or in severe pain. They also expect the call handlers to refrain from commenting in any way whatsoever on clinical issues. In practice, however, it is by no means unusual to find call handlers redefining the boundaries of their roles as they interact with callers. This does not necessarily involve them unilaterally imposing their own *a priori* understandings of their roles and responsibilities. Their actions are inevitably shaped and reshaped in light of the conduct of callers who vary in their knowledge, expectations, objectives, capacities, and so on. Thus, for example, in the case considered above, it is noteworthy that the call handler's 'advice' is occasioned by the caller's question at line 13. Similarly, the varying degrees of empathy shown by the call handler appear to be responsive to, and interrelated with, the ways in which the caller describes and expresses the physical sensations that she is apparently experiencing and the problems that she has encountered in her dealings with other parts of the NHS. The data illustrate the way in which shifting boundaries between nurse and call handler create a space within actual call interactions into which service consumers can move to shape the nature of the service provided. This underlines the fact that the NHS Direct service is not simply 'delivered' by call handlers and nurses and 'consumed' by callers, but is, rather, jointly produced as NHS Direct calls unfold, moment by moment. To understand the production of the NHS Direct service, it is thus necessary to acknowledge and analyse the active reflexive role of the consumer within the production process and to chart the ways in which their conduct impacts upon the delineation of the boundaries of the roles of NHS Direct personnel.

DISCUSSION AND CONCLUSIONS

NHS Direct is a useful vehicle for studying the dynamics of consumption and service interactions in that it represents a combination of call centre and customer-orientated bureaucracy, with all the tensions inherent in both

types of organization. Here were tensions around employees' experience of the work, tensions in the management of it and contradictions in the constructions of callers – at one time, the mythical 'sovereign customer' and, at another, an irate caller failing to appreciate that the call handler was doing their best. Uniquely in call centre organizations, NHS Direct employs highly trained professionals (nurses) and positions them as, to some extent, dependent upon non-clinically trained call handlers. At the same time, call handlers are required to undertake a degree of clinical decision making.

We have suggested that nurses have recourse to a discourse of professionalism in defending degrees of autonomy (both in relation to managerial and to technological imperatives), which, in contrast to the nurses in Wicks's (1998) study of 'traditional' nurses, allows them to give callers and patients the *quality* of care (time, professional expertise, individual attention) they wish to give. Interestingly, it is technology that allows nurses to deal exclusively with one patient at a time in the protected space afforded by the telephone.

Front line call handlers, on the other hand, caught between giving quality 'customer service' and being subject to managerial control, are equipped with a discourse of 'quasi-professionalism'. As our interaction data illustrate, in practice, this leaves them vulnerable to straying into nurses' territory of clinical decision making and to engaging in potentially risky encounters with callers, thereby revealing the tensions around the enhanced levels of discretion (Willmott, 1995) supposedly afforded to these 'grown-up' and 'empowered' front line workers.

Increased bureaucratic control may be responsible for the decline of professional autonomy in a number of public sector services (see, for example, Davies and Kirkpatrick, 1995), but such organizational systems alone cannot account for the nature of service provision. Our call interaction data also show *consumers'* contribution to service production and how, as they invest of themselves in the interaction, they are active participants in the creation of meaning. NHS Direct personnel do not simply produce a service *for* the consumer, since the producer-consumer interaction is also shaped by the response of the consumer/patient. As with many other services (Garcia and Parmar, 1999; Whalen et al., 1988; Zimmerman, 1992), the NHS Direct service and its meaning are jointly produced by professionals *and* consumers. To understand the production of the NHS Direct service, it is thus necessary to acknowledge and analyse the active reflexive role of the consumer within the production process and to chart the ways in which their conduct impacts on the delineation of the boundaries of the roles of NHS Direct personnel.

Korczynski (2002) suggests that du Gay's (1996) idea of the consumer as entrepreneurial does not make sense, since 'the discourse of the enterprising self is one that resides primarily in the sphere of production'. 'What does it mean', he asks rhetorically, 'for a consumer to be enterprising?' (2002: 49). In the context of the service use of NHS Direct, we might say that an enterprising self is constituted by and within processes of production *and* consumption. As mentioned earlier, a number of writers (de Swann, 1990; Lauritzen and Sachs, 2001; Lupton, 1995; Nettleton, 1997) have shown how discourses of personal responsibility for health have led to an internalization of surveillance, which may account for greater rather than lesser demand on professional services. Petersen and Lupton (1996) argue that the pervasiveness and inescapability of risk have become incorporated into health and welfare. They show how health risk is individualized in epidemiological discourses and is constructed as being the personal responsibility of people to manage. A newly emerged 'entrepreneurial self' is expected to live life in a prudent, calculating way and to be ever-vigilant about risks. 'Neo-liberal forms of rule employ technologies to "govern at a distance" by seeking to create localities, entities and persons able to operate a regulated freedom' (1996: xiii). Alongside rights, a 'duties discourse' assigns 'citizens' a whole range of reciprocal responsibilities and obligations, which include a responsibility to be healthy: 'Health has come to be used as a kind of shorthand for signifying the capacity of the modern self to be transformed through the deployment of various rational practices of the self . . . something constantly to be worked on' (1996: xiv). 'Consumption' of the NHS Direct service could be seen as an integral part of this ongoing project of the self.

In practice, the discourses used to define this new health subject, by a government engaged in the social management of health, shift between references to 'customers', 'consumers' and 'citizens' (Elston, 1991; Goode et al., 2004). This is a prime example of Gabriel and Lang's 'unmanageable consumer' and the tendencies of each 'face' to 'mutate into different ones' (1995: 3). However this subject is referred to at different times and whichever agenda different discourses are serving, our research confirms that the 'consumer's' contribution to the joint production of service organizations is crucial.

Notes

1. The project (ref. no. L218252022) was funded by the ESRC and the MRC as part of the Innovative Health Technologies Programme.

2. Statistical data on NHS Direct are drawn from *Hansard* (8 September 2003, Column 237W) and from personal communications with the information officers at the two fieldwork sites.

References

- Ackroyd, S. and Thompson, P. (1999) *Organisational Misbehaviour*. London: Sage.
- Alferoff, A. and Knights, D. (2001) 'Prometheus Unbound? Organizational Knowledge as Resistance and Control', paper presented at the Call Centres and Beyond: The HRM Implication conference, 6 November, King's College, University of London.
- Allen, D. (2000) 'Doing Occupational Demarcation: The "Boundary Work" of Nurse Managers in a District General Hospital', *Journal of Contemporary Ethnography* 29(3): 326–56.
- Arkin, A. (1997) 'Hold the Production Line!', *People Management* (6 Feb.).
- Batt, R. and Moynihan, L. (2001) 'The Viability of Alternative Call Center Production Models', paper presented at the Call Centres and Beyond: The HRM Implication conference, 6 November, King's College, University of London.
- Bolton, Sharon C. (2001) 'Changing Faces: Nurses as Emotional Jugglers', *Sociology of Health and Illness* 23(1): 85–100.
- Culpitt, A. (1999) *Social Policy and Risk*. London: Sage.
- de Swann, A. (1990) *The Management of Normality: Critical Essays in Health and Welfare*. London: Routledge.
- du Gay, P. (1996) *Consumption and Identity at Work*. London: Sage.
- Davies, A. and Kirkpatrick, I. (1995) 'Performance Indicators, Bureaucratic Control and the Decline of Professional Autonomy', in I. Kirkpatrick and M. Martinez Lucio (eds) *The Politics of Quality in the Public Sector: The Management of Change*, pp. 84–107. London: Routledge.
- Deery, S. and Walsh, J. (2001) 'Contracting out and Market-mediated Employment Arrangements: Outsourcing Call Centre Work', paper presented at the Call Centres and Beyond: The HRM Implication conference, 6 November, King's College, University of London.
- DoH (Department of Health) (1997) *The New NHS: Modern, Dependable*, White Paper, CM 3807. London: Stationery Office.
- Elston, M. (1991) 'The Politics of Professional Power: Medicine in a Changing Health Service', in J. Gabe, M. Calnan and M. Bury (eds) *The Sociology of the Health Service*, pp. 58–88. London: Routledge.
- Fernie, S. and Metcalfe, D. (1997) *(Not) Hanging on the Telephone: Payment Systems in the New Sweatshops*. London: Centre for Economic Performance, LSE.
- Gabriel, Y. and Lang, T. (1995) *The Unmanageable Consumer: Contemporary Consumption and its Fragmentations*. London: Sage.
- Garcia, A. and Parmar, P.A. (1999) 'Misplaced Trust: The Collaborative Construction of Doubt in 911 Emergency Calls', *Symbolic Interaction* 22(4): 297–324.
- Giddens, A. (1994) *The Constitution of Society*. Cambridge: Polity Press.
- Goode, J., Hanlon, G., O'Cathain, A., Luff, D., Strangleman, T. and Greatbatch, D. (2004) 'Risk and the Responsible Health Consumer: The Problematics of Entitlement among Callers to NHS Direct', *Critical Social Policy* 24(2): 210–32.

- Greatbatch, D., Hanlon, G., Goode, J., O'Cathain, A., Strangleman, T. and Luff, D. (2005) 'Telephone Triage, Expert Systems and Clinical Expertise', *Sociology of Health and Illness* 27(6): 802–30.
- Hanlon, G. and Jackson, J. (1999) 'Last Orders at the Bar? Competition, Choice and Justice for All', *Oxford Journal of Legal Studies* 19(4): 555–82.
- Hochschild, A. (1983) *The Managed Heart*. Berkeley: University of California Press.
- James, N. (1992) 'Care = Organisation + Physical Labour + Emotional Labour', *Sociology of Health and Illness* 14(4): 489–509.
- Kinnie, N., Purcell, J. and Hutchinson, S. (2000) '"Fun and Surveillance": The Paradox of High Commitment Management in Call Centres', *International Journal of Human Resource Management* 11(5): 967–85.
- Knights, D. and McCabe, D. (1998) '"Ain't Misbehavin"? Opportunities for Resistance within Bureaucratic and Quality Management Innovations', *Sociology* 34(3): 421–37.
- Knights, D. and Odih, P. (2002) '"Big Brother Is Watching You!": Call Centre Surveillance and the Time-disciplined Subject', in G. Crow and S. Heath (eds) *Social Conceptions of Time, Structure and Process in Work and Everyday Life*, pp. 144–61. Basingstoke: Palgrave Macmillan.
- Knowles, E., O'Cathain, A., Morrell, J., Munro, J.F. and Nicholl, J.P. (2002) 'NHS Direct and Nurses: Opportunity or Monotony?', *International Journal of Nursing Studies* 39: 857–66.
- Korczynski, M. (2001) 'The Contradictions of Service Work: Call Centre as Customer-oriented Bureaucracy', in A. Sturdy, I. Grugulis and H. Willmott (eds) *Customer Service: Empowerment and Entrapment*, pp. 60–79. Basingstoke: Palgrave.
- Korczynski, M. (2002) *Human Resource Management in Service Work*. Basingstoke: Palgrave.
- Lankshear, G., Cook, P., Mason, D., Coates, S. and Button, G. (2001) 'Call Centre Employees' Responses to Electronic Monitoring: Some Research Findings', *Work, Employment and Society* 15(3): 595–605.
- Lauritzen, S.O. and Sachs, L. (2001) 'Normality, Risk and the Future: Implicit Communication of Threat in Health Surveillance', *Sociology of Health and Illness* 23(4): 497–516.
- Lupton, D. (1995) *The Imperative of Health: Public Health and the Regulated Body*. London: Sage.
- Nettleton, S. (1997) 'Governing the Risky Self: How to Become Healthy, Wealthy and Wise', in A. Peterson and R. Bunton (eds) *Foucault, Health and Medicine*, pp. 207–22. London: Routledge.
- Petersen, A.R. and Lupton, P. (1996) *The New Public Health: Health and Self in the Age of Risk*. London: Sage.
- Pettinari, C.J. and Jessop, L. (2001) '"Your Ears Become Your Eyes": Managing the Absence of Visibility in NHS Direct', *Journal of Advanced Nursing* 36(5): 668–75.
- Sewell, G. and Wilkinson, B. (1992) 'Someone to Watch over Me: Surveillance, Discipline and the Just-in-time Labour Process', *Sociology* 26(2): 271–91.
- Taylor, P. and Bain, P. (2001) 'Trade Unions, Workers' Rights and the Frontier of Control in UK Call Centres', *Economic and Industrial Democracy* 22(2): 39–66.
- Taylor, P., Mulvey, G., Hyman, J. and Bain, P. (2002) 'Work Organization, Control and the Experience of Work in Call Centres', *Work, Employment and Society* 16(1): 133–50.

- Taylor-Gooby, P., Dean, H., Munro, M. and Parker, G. (1999) 'Risk and the Welfare State', *British Journal of Sociology* 50(2): 177–94.
- Tyler, M. and Taylor, S. (2001) 'Juggling Justice and Care: Gendered Customer Service in the Contemporary Airline Industry', in A. Sturdy, I. Grugulis and H. Willmott (eds) *Customer Service: Empowerment and Entrapment*, pp. 81–98. Basingstoke: Palgrave.
- Whalen, J., Zimmerman, D. and Whalen, M. (1988) 'When Words Fail: A Single-case Analysis', *Social Problems* 35(4): 335–62.
- Wicks, D. (1998) *Nurses and Doctors at Work*. Buckingham: Open University Press.
- Willmott, H. (1995) 'The Odd Couple? Re-engineering Business Processes; Managing Human Relations', *New Technology, Work, and Employment* 10(2): 89–98.
- Zimmerman, D. (1992) 'The Interactional Organisation of Calls for Emergency Assistance', in P. Drew and J. Heritage (eds) *Talk at Work: Interaction in Institutional Settings*, pp. 418–69. Cambridge: Cambridge University Press.
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